Version:1

Bwrdd Iechyd Prifysgol
Betsi Cadwaladr
University Health Board

ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT CPG

OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

ANTENATAL REFERRALS TO THE ANAESTHETIST

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Review	**Purpose of Issue/Description of current changes:** New obstetric anaesthesia written control document for Betsi Cadwaladr University Health Board (West)

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1. Introduction and Purpose

It is critical that patients with certain conditions are referred to the anaesthetist long in advance of delivery, so that a safe anaesthetic and analgesic plan can be formulated.

2. Scope

This guideline is intended as a reference for Obstetricians.

3. Points to consider

Patients with the following conditions should be referred to the anaesthetist antenatally for an anaesthetic assessment :

Difficult Airway

- Markedly receding jaw
- Significant impairment of flexion / extension of neck
- Restricted mouth opening

Cardiac Disease

- Valvular heart disease
- Prosthetic heart valves
- Arrhythmias
- Cardiomyopathy
- Congenital heart disease

Respiratory Disease

- Severe / poorly controlled asthma
- Restrictive pulmonary disease including severe rib cage deformity
- Cystic fibrosis

Musculoskeletal

- Kypho scoliosis
- Serious lumbar spine problems including prolapsed discs, spondylolisthesis or previous trauma
- Systemic lupus erythematosus

Neurological Disease

- Poorly controlled epilepsy
- Multiple sclerosis
- Myasthenia gravis
- Dystrophia myotonica
- Muscular dystrophy
- Spinal cord lesions

Haematological

- Haemophilia
- Other clotting factor deficiency syndromes
- Thrombocytopaenia eg. ITP
- Sickle cell disease
- Jehovah's Witness

Endocrine

- Morbid obesity (see separate guideline for referral procedure)
- Insulin dependent diabetes mellitus
- Poorly controlled thyroid / parathyroid disease (or if large goitre present)
- Addison's disease

Renal Disease

- Dialysis dependent chronic renal failure

Hepatic Disease

- Severe chronic hepatic disease

Immune

- Serious allergies to drugs or other substances eg. latex

Transplant Patients

- All heart, lung, kidney and liver transplant patients

Obstetric problems of anaesthetic concern

- Placenta previa (especially grade 3 and 4 with anterior placenta)
- Severe pre eclampsia

Previous anaesthetic problems

- Difficult or failed intubation
- Allergies or reactions to anaesthetic agents
- Post dural puncture headache
- Serious nerve damage secondary to a regional block
- Patient wishing to discuss anaesthetic / analgesic options following previous problematic general or regional anaesthetic

Medical conditions of specific anaesthetic importance

- Malignant hyperthermia
- Porphyria
- Pseudocholinesterase deficiency (suxamethonium apnoea)

4. References

1. Association of Anaesthetists of Great Britain and Ireland and Obstetric Anaesthetists' Association. *Guidelines for Obstetric Anaesthesia Services.* AAGBI, London,2013.

2. Lewis,G (ed) 2007. Confidential Enquiry into Maternal and Child Health (CEMACH). Saving Mothers' Lives: reviewing maternal deaths to make motherhood safer- 2003-2005. *The Seventh Report on Confidential Enquiries into Maternal Deaths in the UK*. London. CEMACH.